

**SUPERIOR COURT OF CALIFORNIA
COUNTY OF VENTURA**

Tentative Ruling

**2025CUMM037379: JOLIE BRUNS vs LOS ROBLES REGIONAL MEDICAL CENTER,
et al.**

09/02/2026 in Department 21

Motion to Contest/Challenge Good Faith Settlement

Tentative Rulings. Parties and counsel appearing for oral argument should address the tentative decision. Parties may submit on the tentative decision by email, with a copy to all other parties in the matter, to courtroom21@ventura.courts.ca.gov before 8:00 a.m. on the day set for the hearing, with a subject line that includes “SUBMISSION ON TENTATIVE”, Case Number, Title and Party. If fewer than all parties submit on the tentative, the hearing will proceed, and the tentative ruling is subject to change. The clerk cannot advise if you should still appear or not. The decision of whether to appear for a hearing is to be made by the parties and their counsel. (Dept. 21 Rules & Procedures, p. 4, § II.I.)

The following is a statement of the Court’s tentative ruling. The Court may adopt, modify or reject the tentative ruling after hearing. The tentative ruling has no legal effect unless and until adopted by the Court.

Motion: Defendant Dr. Abaya’s Motion Contesting the Good Faith Settlement of Dr. Sun and Los Robles Hospital (opposed)

Tentative Ruling:

Defendant Abaya’s motion to contest application for approval of good faith settlement is DENIED.

Background:

This case involves a wrongful death action arising from medical care provided to Plaintiff’s mother (hereinafter “decedent”) at Los Robles Hospital on February 7, 2022. It is alleged that decedent suffered a fatal cardiopulmonary event after being administered labetalol, despite a contra-indicated history of asthma and COPD. Defendants Los Robles Regional Medical Center and Dr. Justin Sun (a first-year resident) have agreed to pay Plaintiff \$500,000 to fully and finally compromise this claim against them. Dr. Hazel Abaya, the attending physician tasked with supervising Dr. Sun, objects to the settlement, opining that it was a resident who prescribed the subject medication, not Dr. Abaya.

Discussion:

This state has a strong public policy promoting settlements of civil disputes. To this end, parties who settle disputes in good faith are immunized from claims from non-settling parties for equitable indemnity or contribution. (CCP §877.6(c).) There is no precise yardstick for measuring “good faith” of a settlement, but it must harmonize the public policy favoring

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settlements with the competing public policy favoring equitable sharing of costs among tortfeasors or co-obligors. At a minimum, the settlement must be within the reasonable range (aka “ballpark”) of the settling party’s share of liability.

The *settling parties* bear the initial burden of showing that the settlement amounts are “within the reasonable range of the settling tortfeasor’s proportional share of comparative liability for the plaintiff’s injuries.” (*Tech-Bilt, Inc. v Woodward-Clyde & Assoc.* (1985) 38 Cal.3d 488, 499.) Settling parties do this by showing 1) a rough approximation of plaintiff’s total recovery and the settlor’s proportionate liability; 2) the amount paid in settlement; 3) the allocation of settlement proceeds among plaintiffs, and a recognition that a settlor should pay less in settlement than he would if he were found liable after a trial; 4) the financial conditions and insurance policy limits of settling defendants; and 5) the existence of collusion, fraud, or tortious conduct aimed to injure the interests of nonsettling defendants. (*Ibid.*) The settling parties must show the evidentiary basis for any allocations and valuations made so that the court can determine whether they are reasonable. (*L.C. Rudd & Son, Inc. v Superior Court* (1997) 52 Cal.App.4th 742, 748.)

Finally, practical considerations obviously require that the evaluation be made on the basis of information available at the time of settlement. “[A] defendant’s settlement figure must not be grossly disproportionate to what a reasonable person, at the time of the settlement, would estimate the settling defendant’s liability to be.” (*Torres v. Union Pacific R.R. Co.* (1984) 157 Cal.App.3d 499, 509 [203 Cal.Rptr. 825].)

(*Tech-Bilt, Inc. v. Woodward-Clyde & Associates* (1985) 38 Cal.3d 488, 499.)

With that in mind, the settlement is discussed below.

1) A rough approximation of plaintiff’s total recovery and the settlor’s proportionate liability:

The settling parties must include in their agreement a joint allocation of the amount. Such allocation is essential to a “good faith” determination because without it there is no way to compute the set-off to which the nonsettling defendants will be entitled against whatever judgment is rendered. (*L.C. Rudd & Son, Inc. v. Superior Court* (1997) 52 Cal.App.4th 742, 750; *Alcal Roofing & Insulation v. Superior Court* (1992) 8 Cal.App.4th 1121, 1124.) Absent an allocation, the trial court must allocate post-trial “in the manner which is most advantageous to the nonsettling defendants.” (*Dillingham Const. N.A., Inc. v. Nadel Partnership, Inc.* (1998) 64 Cal.App.4th 264, 288.) The settling parties here elected not to include any allocation as between them the hospital and Dr. Sun, or as between economic and noneconomic damages. As such, Dr. Abaya will be allowed to seek the most favorable Prop 51 allocation she can at trial.

This Court can take judicial notice of the fact that MICRA places a cap on wrongful death non-economic damages in this case at \$600,000. (See Civil Code §3333.2(g).) There is also a cap for the survival action of \$430,000. (*Id.*) Where several health care providers are liable for a plaintiff’s noneconomic damages, the MICRA cap limits a plaintiff’s recovery against all liable health care providers collectively. As explained in *Rashidi v. Moser* (2014) 60 Cal.4th 718 (at 729): “the cap performed its role in the settlement arena by providing Cedars–Sinai with a limit on its exposure to liability. Had Moser established any degree of fault on his codefendants’ part

at trial, he would have been entitled to a proportionate reduction in the capped award of noneconomic damages.”

As to the settlors proportion of liability, assuming Dr. Abaya’s job was to observe and supervise Dr. Sun, settlors have a strong argument that it was unlikely that Dr. Sun’s decision to prescribe Labetalol to decedent went unnoticed. The evidence suggests that Dr. Abaya was in close proximity to decedent throughout the process and was more likely than not aware of the decision to administer Labetalol. Dr. Sun testified that the labetalol would have been ordered after a discussion with Dr. Abaya, and would involve teaching. (Sun depo, p. 17-18.) Additionally, Dr. Avedikian testified that he was present when Dr. Sun and Dr. Abaya discussed administering labetalol to decedent and that it was Dr. Abaya who instructed Dr. Sun to put in the order for the labetalol. (Avedikian depo, p. 22-23.)

Here, the settling defendants paying \$500,000 – when their theoretical maximum non-economic exposure was \$1,030,000 – reflects a settlement of approximately 50% of the total risk. In most instances, this would qualify as sufficiently “in the ballpark” to be considered good faith.

Overall, based on the testimony of Dr. Sun and Dr. Avedikian, the rough proportion of Plaintiff’s recovery is proportionate with Dr. Sun and Los Robles’s likely proportionate share of liability. These factors weigh in favor of finding the settlement to be in good faith.

2) The amount paid in settlement:

The settlement amount is \$500,000. This sum is not significantly disproportionate, as is explained above.

3) The allocation of settlement proceeds among plaintiffs, and a recognition that a settlor should pay less in settlement than he would if he were found liable after a trial:

There is only one Plaintiff, so the allocation issue is inapplicable. The recognition that a settlor should pay less in settlement supports finding that the settlement was made in good faith.

4) The financial conditions and insurance policy limits of settling defendants:

Settling Defendants’ financial conditions and insurance policy limits are not disclosed; however, in this case, the financial condition and insurance policy limits are of de minimis importance, in light of the amount of the settlement, the potential damages, and proportionate share of liability.

5) The existence of collusion, fraud, or tortious conduct aimed to injure the interests of nonsettling defendants:

There is no evidence of collusion, fraud, or tortious conduct. The settlement appears fair and reasonable.

Conclusion: The papers show that the settlement was made in good faith, and the settlement amount is within the reasonable range of the settling parties proportional share of liability for the

alleged injuries and damages. Settling Defendants have met their initial burden of showing good faith. Dr. Abaya has not met her burden of proof showing lack of good faith. Dr. Abaya's motion to oppose good faith settlement is DENIED.